

Exploring Attachment Styles, Self-Compassion, and Emotional Distress

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Anxious and avoidant attachment styles, both insecure attachment styles in adulthood, have been consistently linked to higher levels of emotional distress, such as depression and anxiety. Self-compassion, or the tendency to be kind to oneself during times of difficulty, has been hypothesized as a potential buffer against this distress.

The present study uses archival data from Joeng et al. (2017) to explore three research questions: whether anxious and avoidant attachment predict depression, whether gender and university group affect anxiety levels, and whether males and females differ in self-compassion. We hypothesized that both attachment styles would positively predict depression. The remaining two questions were treated as exploratory.

Method

This project is an archival analysis of data from Joeng et al. (2017), which examined the relationships between attachment styles, self-compassion, and emotional distress. The data were collected from 473 college students recruited from three South Korean universities via university email lists, websites, and psychology classes. Of the participants, 288 (61%) were male and 185 (39%) were female, with a mean age of 25.26 years ($SD = 3.78$).

Measures

Attachment styles. Anxious and avoidant attachment were measured using the 18-item Anxious Attachment and 18-item Avoidant Attachment subscales of the Korean version of the Experiences in Close Relationships-Revised Scale (ECR-R). Items were rated on a 7-point Likert scale (1 = strongly disagree to 7 = strongly agree). Higher scores indicate greater anxious or

avoidant attachment. In the current study, Cronbach's $\alpha = 0.94$ for anxious attachment and $\alpha = 0.79$ for avoidant attachment.

Self-compassion. Self-compassion was measured using the Korean version of the Self-Compassion Scale (SCS). This 26-item scale asks participants to rate how they typically act toward themselves in difficult times on a 5-point Likert scale (1 = almost never to 5 = almost always). Higher scores indicate greater self-compassion. Cronbach's $\alpha = 0.90$ in the current study.

Depression. Depression was measured using the Korean version of the Center for Epidemiological Studies-Depression Scale (CES-D), which consists of 20 items assessing depressive symptoms over the past week on a 4-point scale (0 = ≤ 1 day to 3 = most or all of the time). Higher scores indicate greater depressive symptoms. Cronbach's $\alpha = 0.92$ in the current study.

Anxiety. Anxiety was measured using the Trait Anxiety subscale of the Korean version of the State-Trait Anxiety Inventory (STAI-T), which consists of 20 items rated on a 4-point scale (1 = almost never to 4 = almost always). Higher scores indicate greater trait anxiety. Cronbach's $\alpha = 0.94$ in the current study.

Results

Across the full sample ($N = 473$), participants reported a mean anxious attachment score of 3.86 ($SD = 0.77$), a mean avoidant attachment score of 3.05 ($SD = 1.04$), and a mean depression score of 1.78 ($SD = 0.47$).

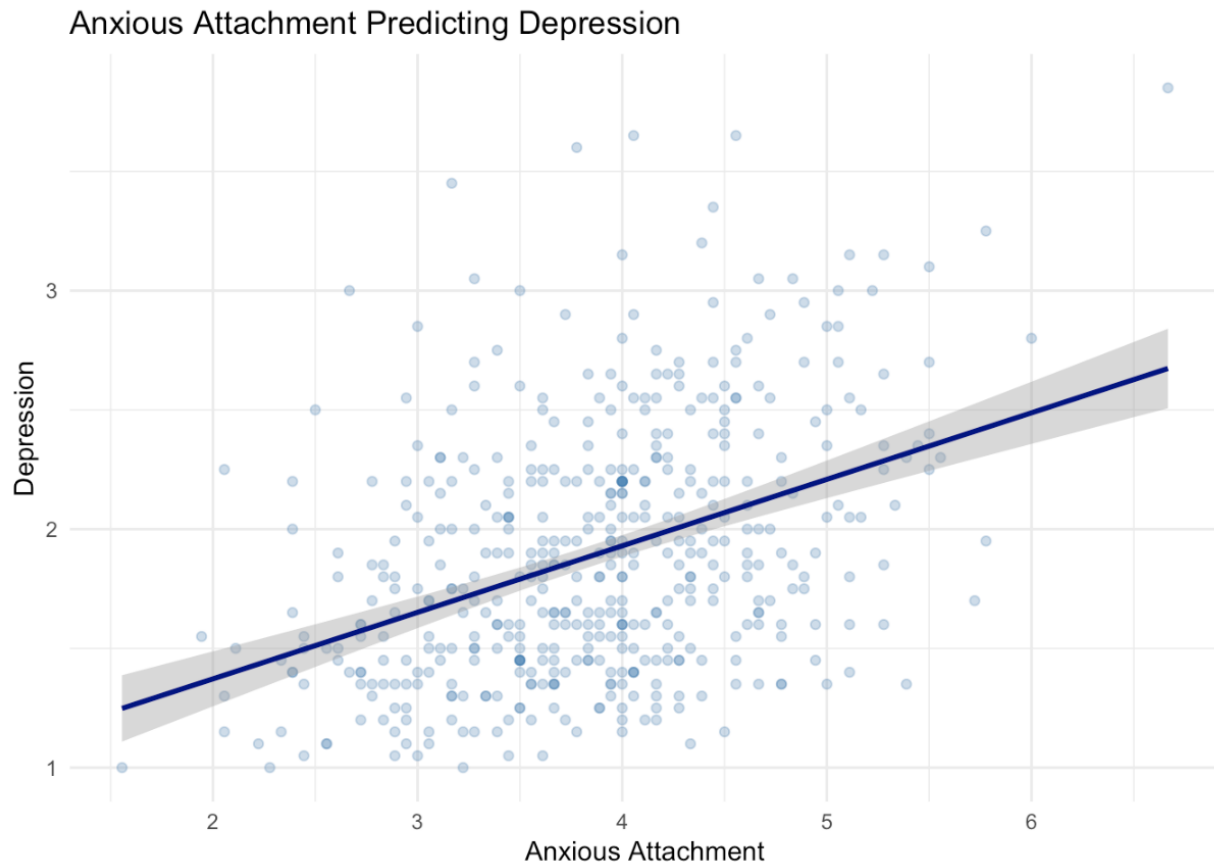
Regression

A multiple linear regression was conducted to examine whether anxious attachment and avoidant attachment predicted depression. The overall model was statistically significant, $F(2, 470) = 103.7, p < .001, R^2 = .306$, indicating that attachment styles explained approximately 30% of the variance in depression. Both anxious attachment ($B = 0.18, SE = 0.03, t(470) = 6.26, p < .001$) and avoidant attachment ($B = 0.21, SE = 0.02, t(470) = 9.86, p < .001$) were significant positive predictors of depression, suggesting that higher levels of both attachment styles were associated with greater depression. As shown in Figure 1, depression scores increased with higher anxious attachment levels.

Several assumptions were examined. A Shapiro-Wilk test indicated a violation of normality ($W = 0.969, p < .001$); however, given the large sample size ($n = 473$), this violation is unlikely to meaningfully affect the results. Mild heteroscedasticity was also detected, $\chi^2(1) = 4.44, p = .035$, and both predictors showed slight non-linearity ($p < .05$). Results should therefore be interpreted with some caution.

Figure 1

Relationship Between Anxious Attachment and Depression.



Note. The line represents the fitted linear regression line with a 95% confidence interval.

Two-way ANOVA

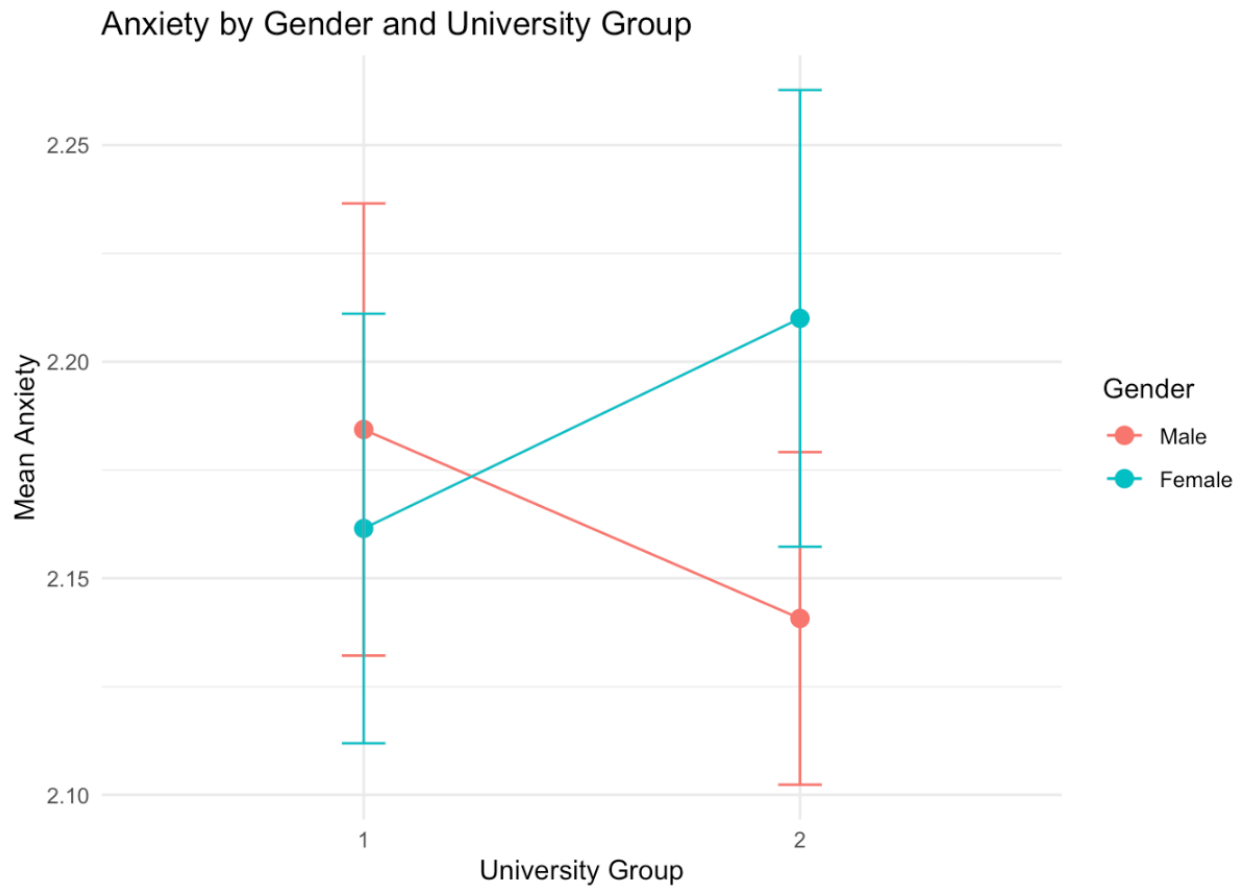
A two-way analysis of variance (ANOVA) was conducted to examine whether gender and university group affected anxiety levels, and whether the effect of university group differed by gender. Descriptive statistics indicated that anxiety levels were similar across all four groups: males in group 1 ($n = 99$, $M = 2.18$, $SD = 0.52$), males in group 2 ($n = 189$, $M = 2.14$, $SD = 0.53$), females in group 1 ($n = 100$, $M = 2.16$, $SD = 0.50$), and females in group 2 ($n = 85$, $M = 2.21$, $SD = 0.49$).

The ANOVA revealed no significant main effect of gender, $F(1, 469) = 0.34, p = .561$, no significant main effect of university group, $F(1, 469) = 0.01, p = .911$, and no significant interaction between gender and group, $F(1, 469) = 0.87, p = .351$. These results suggest that anxiety levels did not differ as a function of gender, university group, or their interaction. As shown in Figure 2, mean anxiety levels were similar across groups for both males and females.

Regarding assumptions, a Shapiro-Wilk test indicated a mild violation of normality ($W = 0.988, p = .001$); however, this is unlikely to meaningfully affect the results given the large sample size. Levene's test indicated that the assumption of homogeneity of variance was met, $F(3, 469) = 0.43, p = .731$.

Figure 2

Mean Anxiety by Gender and University Group.



Note. Error bars represent standard errors.

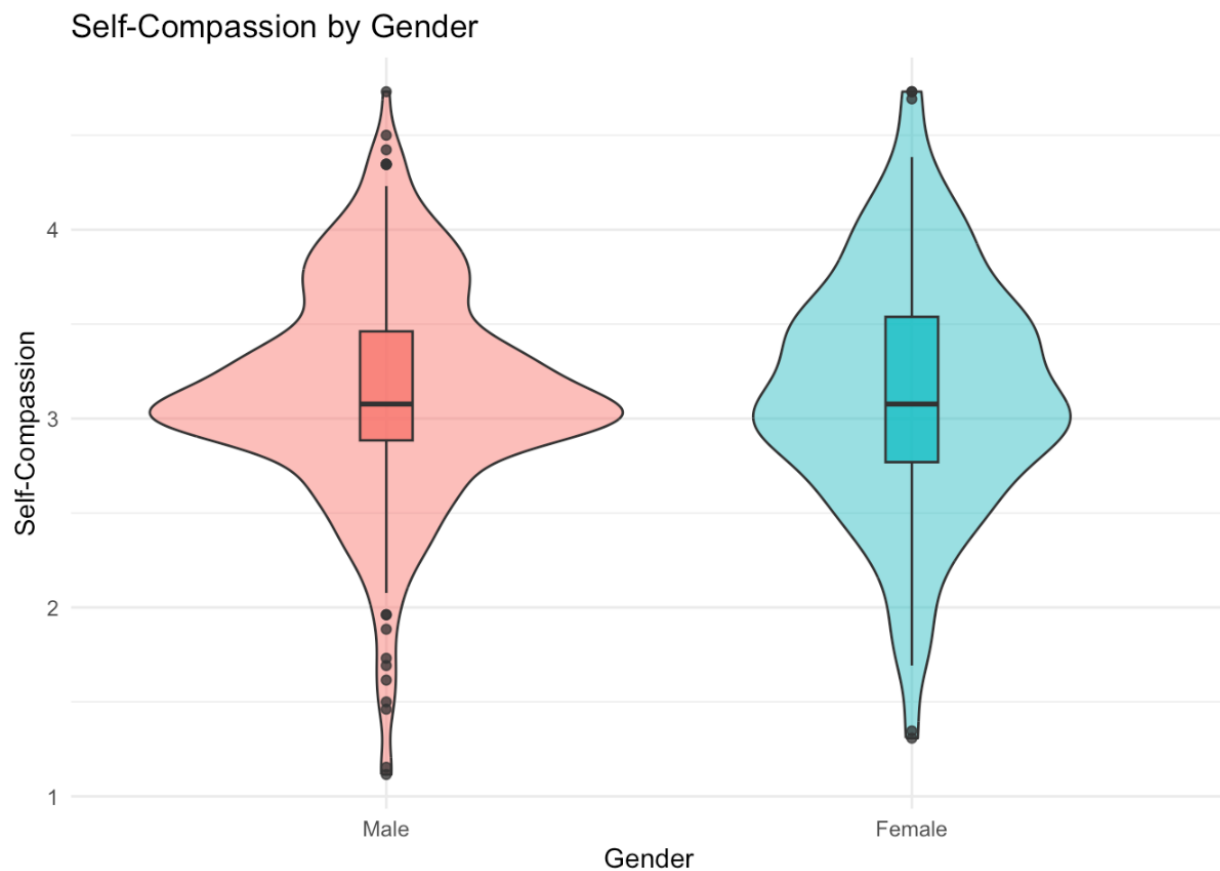
Independent Samples T-test

An independent samples *t*-test was conducted to examine whether males and females differed in self-compassion. Descriptive statistics indicated that males ($n = 288$, $M = 3.14$, $SD = 0.57$) and females ($n = 185$, $M = 3.14$, $SD = 0.63$) reported nearly identical levels of self-compassion.

Levene's test indicated that the assumption of homogeneity of variance was met, $F(1, 471) = 3.56, p = .060$. The t -test revealed no significant difference between males and females in self-compassion, $t(471) = -0.055, p = .956$, 95% CI $[-0.11, 0.11]$. These results suggest that self-compassion levels do not differ by gender in this sample. As shown in Figure 3, the distribution of self-compassion scores was nearly identical for males and females.

Figure 3

Self-Compassion Scores by Gender.



Note. Boxplots show the distribution of self-compassion scores for male and female participants.

Discussion

This study investigated three research questions using archival data from Korean college students. As predicted in our first hypothesis, both anxious and avoidant attachment were significant positive predictors of depression. The overall model explained about 30% of the variance in depression scores. These results are consistent with previous research demonstrating that insecure attachment is related to higher levels of emotional distress. Individuals with an anxious attachment style may have persistent hyperactivation of distress, marked by constant worry about rejection and other reassurance needs that are hard to meet. Individuals with an avoidant attachment style may suppress emotional needs and distance themselves from social support, both of which could worsen depressive symptoms over time.

For the second and third research questions, gender and university group did not significantly affect anxiety levels, and males and females did not differ in self-compassion. These null findings indicate that anxiety and self-compassion were relatively comparable across demographic groups in this sample. This may reflect the homogeneous nature of the sample, as all participants were Korean college students, which could limit variability across groups. Cultural factors, such as the influence of Confucian values on emotional expression, may also shape how self-compassion is experienced similarly across genders in this context.

Several limitations should be noted. First, the data are cross-sectional, meaning causality cannot be inferred. Second, mild violations of normality and homoscedasticity were detected in the regression model, suggesting results should be interpreted with caution. Future research could address these limitations by using longitudinal designs and larger, more diverse samples to examine whether these relationships generalize across cultures and age groups.